



LABORATORY REPORT



Name : Mrs. SWETAL SHAH	Sex/Age : Female / 44 Years	Case ID : 50500100197
Ref. By : Dr. Atul P Munshi MD, DGO	Dis. At :	Pt. ID : 411858
Bill. Loc. :		Pt. Loc :
Reg Date and Time : 01-May-2025 09:48	Sample Type :	Mobile No. : 9601756054
Sample Date and Time :	Sample Coll. By : NSRL	Ref Id1 :
Report Date and Time :	Acc. Remarks :	Ref Id2 :

Abnormal Result(s) Summary

Test Name	Result Value	Unit	Reference Range
Haemogram (CBC)			
Total WBC Count	11140	/μL	4000.00 - 10000.00
Neutrophil	78.2	%	40.00 - 70.00
Lymphocyte	14.5	%	20.00 - 40.00
Neutrophil	8711	/μL	2000.00 - 7000.00
Neut/Lympho Ratio (NLR)	5.39		0.78 - 3.53
Platelet Count	462000	/μL	150000.00 - 410000.00
Urine Examination			
Leucocytes (ESTERASE)	+		Negative
Protein	(+)		Negative
Blood	+++		Negative
Leucocyte	6	/HPF	Nil
RBC (Isomorphic)	44	/HPF	Nil
S.G.P.T.	44.88	U/L	< 34
Sodium	135.00	mmol/L	136 - 145

Abnormal Result(s) Summary End

Note:(LL-VeryLow,L-Low,H-High,HH-VeryHigh ,A-Abnormal)

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Bill. Loc. :		Pt. Loc :
Reg Date and Time : 01-May-2025 09:48	Sample Type : Plasma Citrate	Mobile No. : 9601756054
Sample Date and Time : 01-May-2025 09:49	Sample Coll. By : NSRL	Ref Id1 :
Report Date and Time : 01-May-2025 11:03	Acc. Remarks : -	Ref Id2 :

TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
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HAEMATOLOGY INVESTIGATIONS

ACTIVATED PARTIAL THROMBOPLASTIN TIME

Test for APTT (Photooptical clot detection)	24.2	seconds	23.6 - 35.8
Control For APTT	29.7	seconds	

Interpretation :

aPTT test is used to measure and evaluate all the clotting factors involved in intrinsic and common coagulation pathways. It measures the time for clot formation after addition of calcium and phospholipid emulsion to plasma sample.

A prolonged aPTT result may indicate the following - Congenital deficiencies of intrinsic system clotting factors, Von Willebrand disease, Hypofibrinogenemia, Liver cirrhosis, Vitamin K deficiency, Disseminated intravascular coagulation (DIC), Heparin therapy, other inhibitors such as lupus anticoagulant and anticardiolipin antibodies, systemic lupus erythematosus, rheumatoid arthritis, and chronic glomerulonephritis.

A shortened aPTT result may indicate the following Early stages of DIC, immediately after acute haemorrhage, An acute-phase response leading to high factor VIII levels

Note:(LL-VeryLow,L-Low,H-High,HH-VeryHigh ,A-Abnormal)

Dr Harveen Bhusari

MD Pathology (G-62229)

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**Neuberg Diagnostics Private Limited**

Laboratory : "KEDAR" Opposite Krupa Petrol Pump, Near Parimal Garden,
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Bill. Loc. :		Pt. Loc. :
Reg Date and Time : 01-May-2025 09:48	Sample Type : Serum	Mobile No. : 9601756054
Sample Date and Time : 01-May-2025 09:49	Sample Coll. By : NSRL	Ref Id1 :
Report Date and Time : 01-May-2025 11:45	Acc. Remarks : -	Ref Id2 :

TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
Leutinizing Hormone CMIA	3.24	mIU/mL	Follicular phase 1.80 to 11.78 mIU/mL Mid cycle peak 7.59 to 89.08 mIU/mL Leuteal phase 0.56 to 14.00 mIU/mL Post menopausal 5.16 to 61.99 mIU/mL	

INTERPRETATIONS:

Useful as an adjunct in the evaluation of menstrual irregularities, Evaluating patients with suspected hypogonadism, Predicting ovulation, Evaluating infertility, Diagnosing pituitary disorders.

In both males and females, primary hypogonadism results in an elevation of basal follicle-stimulating hormone (FSH) and luteinizing hormone (LH) levels.

Postmenopausal LH levels are generally >40 IU/L. (Note: FSH is the preferred test to confirm menopausal status.)

FSH and LH are generally elevated in:

Primary gonadal failure, Complete testicular feminization syndrome, Precocious puberty (either idiopathic or secondary to a central nervous system lesion), Menopause, Primary ovarian hypodysfunction in females, Polycystic ovary disease in females, Primary hypogonadism in males

LH is decreased in Primary ovarian hyperfunction in females, Primary hypergonadism in males.

FSH and LH are both decreased in failure of the pituitary or hypothalamus

CAUTIONS:

No clinically significant cross-reactivity has been demonstrated with follicle-stimulating hormone, thyroid stimulating hormone, or human chorionic gonadotropin. Some patients who have been exposed to animal antigens, either in the environment or as part of treatment or imaging procedures, may have circulating antianimal antibodies present. These antibodies may interfere with the assay reagents to produce unreliable results

Note:(LL-VeryLow,L-Low,H-High,HH-VeryHigh ,A-Abnormal)

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Follicle Stimulating Hormone 10.34 mIU/mL

Normal menstruating females:
3.03-8.08 Follicular phase
2.55-16.69 Mid cycle peak
1.38-5.47 Luteal phase
Post Menopausal females:
26.72-133.41

INTERPRETATIONS:

Useful for an adjunct in the evaluation of menstrual irregularities, Evaluating patients with suspected hypogonadism , Predicting ovulation , Evaluating infertility & Diagnosing pituitary disorders.

In both males and females, primary hypogonadism results in an elevation of basal follicle-stimulating hormone (FSH) and luteinizing hormone (LH) levels.

FSH and LH are generally elevated in:

-Primary gonadal failure , Complete testicular feminization syndrome, Precocious puberty (either idiopathic or secondary to a central nervous system lesion),

Menopause (postmenopausal FSH levels are generally >40 IU/L), Primary ovarian hypofunction in females , Primary hypogonadism in males.

Normal or decreased FSH in:

-Polycystic ovary disease in females

FSH and LH are both decreased in failure of the pituitary or hypothalamus.

CAUTIONS:

Some patients who have been exposed to animal antigens, either in the environment or as part of treatment or imaging procedures, may have circulating antianimal antibodies present. These antibodies may interfere with the assay reagents to produce unreliable results.

Note:(LL-VeryLow,L-Low,H-High,HH-VeryHigh ,A-Abnormal)

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Sample Date and Time : 01-May-2025 09:49	Sample Coll. By : NSRL	Ref Id1 :
Report Date and Time : 01-May-2025 11:46	Acc. Remarks : -	Ref Id2 :

TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
HCV antibody CMIA	0.15	S/CO	0 - <1 Non Reactive 1 - 3 Indeterminate > 3 Positive	

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DCP.(G-17008)

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Bill. Loc. :		Pt. Loc :
Reg Date and Time : 01-May-2025 09:48	Sample Type : Whole Blood EDTA	Mobile No. : 9601756054
Sample Date and Time : 01-May-2025 09:49	Sample Coll. By : NSRL	Ref Id1 :
Report Date and Time : 01-May-2025 10:16	Acc. Remarks : -	Ref Id2 :

TEST	RESULTS	UNIT	BIOLOGICAL REF. INTERVAL	REMARKS
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HAEMOGRAM REPORT

HB AND INDICES

Haemoglobin	12.1	G%	12.00 - 15.00
RBC (Electrical Impedance)	4.15	millions/cumm	3.80 - 4.80
PCV(Calc)	36.94	%	36.00 - 46.00
MCV (RBC histogram)	89.0	fL	83.00 - 101.00
MCH (Calc)	29.2	pg	27.00 - 32.00
MCHC (Calc)	32.8	gm/dL	31.50 - 34.50
RDW (RBC histogram)	14.00	%	11.00 - 16.00

TOTAL AND DIFFERENTIAL WBC COUNT (Flowcytometry)

Total WBC Count	H 11140	/μL	4000.00 - 10000.00
Neutrophil	H 78.2	%	EXPECTED VALUES 40.00 - 70.00
Lymphocyte	L 14.5	%	20.00 - 40.00
Eosinophil	2.2	%	1.00 - 6.00
Monocytes	4.9	%	2.00 - 10.00
Basophil	0.2	%	0.00 - 2.00

	H	[Abs]	EXPECTED VALUES
Neut/Lympho Ratio (NLR)	H 5.39		0.78 - 3.53

PLATELET COUNT (Optical)

Platelet Count	H 462000	/μL	150000.00 - 410000.00
MPV	9.10	fL	6.5 - 12
PDW	15.7	fL	15 - 17

SMEAR STUDY

RBC Morphology	Normocytic Normochromic RBCs.
WBC Morphology	Leucocytosis, Neutrophilia

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[Signature]

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Sample Date and Time : 01-May-2025 09:49	Sample Coll. By : NSRL	Ref Id1 :
Report Date and Time : 01-May-2025 10:16	Acc. Remarks : -	Ref Id2 :

Platelet

Thrombocytosis

Parasite

Malarial Parasite not seen on smear.

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Sample Date and Time : 01-May-2025 09:49	Sample Coll. By : NSRL	Ref Id1 :
Report Date and Time : 01-May-2025 11:05	Acc. Remarks : -	Ref Id2 :

TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
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HAEMATOLOGY INVESTIGATIONS

BLOOD GROUP AND RH TYPING (Erythrocyte Magnetized Technology) (Both Forward and Reverse Group)

ABO Type	O
Rh Type	Positive

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Bill. Loc. :		Pt. Loc :
Reg Date and Time : 01-May-2025 09:48	Sample Type : Plasma Fluoride R,Serum	Mobile No. : 9601756054
Sample Date and Time : 01-May-2025 09:49	Sample Coll. By : NSRL	Ref Id1 :
Report Date and Time : 01-May-2025 11:45	Acc. Remarks : -	Ref Id2 :

TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
Plasma Glucose - R <i>Photometric, Hexokinase</i>	106.23	mg/dL	70.00 - 160.00	
S.G.P.T. <i>NADH (Without P-5-P)</i>	H 44.88	U/L	< 34	
Urea <i>Urease</i>	15.06	mg/dL	14.98 - 40.01	
Creatinine <i>Jaffe - Kinetic</i>	0.78	mg/dL	0.5 - 1.1	
Sodium <i>ISE</i>	L 135.00	mmol/L	136 - 145	
Potassium <i>ISE</i>	4.00	mmol/L	3.5 - 5.1	

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Bill. Loc. :		Pt. Loc. :
Reg Date and Time : 01-May-2025 09:48	Sample Type : Plasma Citrate	Mobile No. : 9601756054
Sample Date and Time : 01-May-2025 09:49	Sample Coll. By : NSRL	Ref Id1 :
Report Date and Time : 01-May-2025 11:03	Acc. Remarks : -	Ref Id2 :

TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
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HAEMATOLOGY INVESTIGATIONS

PROTHROMBIN TIME

Carried out with ISI=0.99

PT Test	10.6		10.1 - 12.9
Control (MNPT)	11.6	seconds	
INR	0.91		0.85 - 1.15

Interpretation :

The prothrombin time (PT) and international normalized ratio (INR) are measures of the extrinsic pathway of coagulation.

The INR is used only for patients on stable oral anticoagulant therapy. It makes no significant contribution to the diagnosis or treatment of patients whose PT is prolonged for other reasons. Suggested INR range=2.0 - 3.0 for standard doses therapy for treatment or prophylaxis of venous thrombosis or pulmonary or systemic embolus. Suggested INR range = 2.5 - 3.5 for high-risk patients with mechanical heart valves

Increased PT times may be due to:

Factor deficiencies(X , II , V , I), Coumadin (warfarin) therapy, Liver Diseases (Bile duct obstruction, Cirrhosis , Hepatitis), Hemorrhagic Disease of the newborn, DIC, Malabsorption, Fibrinolysis, Vitamin K deficiency.

Interference in PT/INR

Alcohol, antibiotics, aspirin, cimetidine, thrombin Inhibitors (Increase PT) Barbiturates, oral contraceptives, hormone-replacement therapy (HRT), and vitamin K (Decrease PT)

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Bill. Loc. :		Pt. Loc :
Reg Date and Time : 01-May-2025 09:48	Sample Type : Spot Urine	Mobile No. : 9601756054
Sample Date and Time : 01-May-2025 09:49	Sample Coll. By : NSRL	Ref Id1 :
Report Date and Time : 01-May-2025 11:10	Acc. Remarks : -	Ref Id2 :

TEST	RESULTS	UNIT	BRI	TEST	RESULTS	UNIT	BRI
URINE EXAMINATION (STRIP METHOD AND AUTOMATED IMAGE EVALUATION)							
<u>Physical and Chemical examination</u>							
Colour	Yellow			Glucose	Negative		Negative
Appearance	Light turbid			Ketone Bodies Urine	Negative		Negative
Sp.Gravity	1.024		1.003 - 1.035	Urobilinogen	Negative		Negative
pH	5.50		4.6 - 8	Bilirubin	Negative		Negative
Leucocytes (ESTERASE)	+		Negative	Nitrite	Negative		Negative
Protein	(+)		Negative	Blood	+++		Negative
<u>Automated Microscopy</u>							
Leucocyte	6	/HPF	Nil	Granular Cast	Nil	/HPF	Nil
Leucocyte Clumps	Nil	/HPF		RBC cast	Nil	/HPF	Nil
RBC (Isomorphic)	44	/HPF	Nil	WBC Cast	Nil	/HPF	Nil
RBC (Dysmorphic)	Nil	/HPF	Nil	Fatty Cast	Nil	/HPF	Nil
<u>Epithelial cells</u>				Mixed Cast	Nil	/HPF	Nil
Squamous EC	Present +	/HPF	Present	<u>Crystals</u>			
NonSquamous EC	Nil	/HPF		Crystals	6	/HPF	Nil
Transitional EC	Nil	/HPF		Calcium oxalate Monohydrate	Nil	/HPF	Nil
Renal Tubular EC	Nil	/HPF		Calcium oxalate Dihydrate	Nil	/HPF	Nil
<u>Bacteria</u>				Triple phosphate	Nil	/HPF	Nil
Bacteria	5434	/μL	Nil	Uric acid	Nil	/HPF	Nil
Bacteria Rods	Nil	/μL	Nil	Leucine	Nil	/HPF	Nil
Bacteria Cocci	5430	/μL	Nil	Amorphous	84	/HPF	Nil
Yeast	Nil	/μL	Nil	Trichomonas	Nil	/μL	Nil
<u>Cast</u>							
Hyaline Cast	2	/HPF	Nil				
Pathological Cast	3	/HPF	Nil				
Hyaline granular cast	Nil	/HPF					

Note:(LL-VeryLow,L-Low,H-High,HH-VeryHigh ,A-Abnormal)

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Sample Date and Time : 01-May-2025 09:49	Sample Coll. By : NSRL	Ref Id1 :
Report Date and Time : 01-May-2025 11:46	Acc. Remarks : -	Ref Id2 :

TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
Hepatitis B Surface Antigen CMIA	0.22	S/CO	<1-Non-Reactive >=1-Reactive	- HBsAg is the earliest marker of acute HBV infection which typically becomes detectable 2-3 months (as early as 14 days) after infection. When symptoms of hepatitis are present, most patients have detectable HBsAg although few patients will have neither HBsAg nor anti-HBs and anti-HBc IgM is the only marker of acute HBV infection (Core window). HBsAg typically persists for 12-20 weeks after onset of symptoms in uncomplicated HBV infection and disappears followed by a small but variable gap with onset of anti-HBs (Seroconversion). - Detection of HBsAg beyond 06 months defines chronic HBV infection or a chronic carrier state. Chronic HBV infection is seen in 1-2% of adults and adolescents following acute HBV infection, 5-10% of immunocompromised individuals and upto 80% of neonates. The chronic carrier state of HBV shows only persistent HBsAg in the serum without any other HBV marker or evidence of liver injury. - Hepatitis B vaccination does not cause a positive HBsAg result. Quantitation or Titer of HBsAg is of no clinical value. • Presence of anti-HBs without detectable HBsAg indicates recovery from acute HBV infection, absence of infectivity and immunity against future HBV infection. • HBsAg test is carried out with Chemiluminescence with monoclonal anti-HBs for the detection of HBsAg. HBsAg assays are routinely used to aid in the diagnosis of suspected hepatitis B viral (HBV) infection and to monitor the status of infected individuals.

CAUTION:

Specimens from patients who have received preparations of mouse monoclonal antibodies for diagnosis or therapy may contain human anti-mouse antibodies (HAMA). Such specimens may show either falsely elevated or depressed values when tested with assay kits which employ mouse monoclonal antibodies. Additional clinical or diagnostic information may be required to determine patient status. • All initial reactive specimens are subjected to further testing by one or two additional methods and final report is issued in accordance with the same. Repeat reactive specimens MUST be confirmed by any combination of the confirmatory tests (e.g. HBsAg neutralization test, Other HBV markers & LFT and HBV DNA by PCR method). • If the HBsAg results are inconsistent with clinical evidence, additional testing is suggested to confirm the result. • For diagnostic purposes, results should be used in conjunction with patient history and other hepatitis markers for diagnosis of acute or chronic infection. Samples containing particulate matter or red blood cells must be centrifuged prior to running the assay. • Specimens from heparinized patients may be partially coagulated and erroneous results could occur due to the presence of fibrin. To prevent this phenomenon, draw the specimen prior to heparin therapy.

HbsAg value ranges between 1.0 to 10.0 index should be considered as indeterminate and advised to repeat with new fresh sample.

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MC-6136

LABORATORY REPORT



Name : Mrs. SWETAL SHAH	Sex/Age : Female / 44 Years	Case ID : 50500100197
Ref. By : Dr. Atul P Munshi MD, DGO	Dis. At :	Pt. ID : 411858
Bill. Loc. :		Pt. Loc :
Reg Date and Time : 01-May-2025 09:48	Sample Type : Serum	Mobile No. : 9601756054
Sample Date and Time : 01-May-2025 09:49	Sample Coll. By : NSRL	Ref Id1 :
Report Date and Time : 01-May-2025 11:46	Acc. Remarks : -	Ref Id2 :

TEST	RESULTS	UNIT	BIOLOGICAL REF RANGE	REMARKS
HIV I & II (p24 antigen and antibody combo)				
HIV I & II CMIA	0.06	S/CO	0-1 Non Reactive 1.1-5 Indeterminate > 5 Reactive	

- HIV Ag/Ab Combo is fourth generation, automated Chemiluminescence (ECLIA/CMIA/CLIA) test designed for antigen & antibody detection of HIV-1 & 2 and includes recombinant antigens and synthetic peptides derived from native TMP sequences to cover the genetic diversity across HIV-1 groups M & O and between HIV-1 and HIV-2. Serologic studies indicate that although HIV-1 and HIV-2 share multiple common epitopes in their core antigens, the envelope glycoproteins are much less cross-reactive. Antibodies elicited against the TMP (or portions of the TMP) of a viral strain within one group or type may react well, poorly, or not at all with the TMP (or portions of the TMP) from a viral strain of a different group or type. The HIV Ag/Ab Combo also detects core protein p24, a marker of early infection before seroconversion.,
- The advantage of the fourth generation combo kit is increased sensitivity of detection of HIV infection due to significant reduction in the diagnostic window even prior to seroconversion.
- False positive result: Auto-immune diseases, multiple pregnancies, multiple transfusions, antibody to Class II HLA Ag (HLA-DR4), hypergammaglobulinemia, antipolystyrene antibodies, chronic alcoholics, hepatitis, HBV immunization, technical error etc. Others
- False negative result: Infected but not yet seroconverted, window period, late stage disease (immune collapse) and technical error

CAUTION

- As per local regulatory guidelines, all initial reactive results by primary method are subjected to further testing by one or two additional methods (Strategies II & III, NACO guidelines 2007) and final report is issued in accordance with the same. Repeat reactive specimens MUST be confirmed by any combination of the confirmatory tests as recommended by NACO.
- Various screening kits available for detection of HIV antibody or antibody & antigen combination show discordant results due to variable sensitivity and inherent limitations of the technique. If the assay results are inconsistent with clinical evidence, additional testing is suggested to confirm the result.
- Specimens from patients who have received reparations of mouse monoclonal antibodies for diagnosis or therapy may contain human anti-mouse antibodies (HAMA). Such specimens may show either falsely elevated or depressed values when tested with assay kits that employ mouse monoclonal antibodies. Additional clinical or diagnostic information may be required to determine patient status.
- Heterophilic antibodies in human serum can react with reagent immunoglobulins, interfering with in vitro immunoassays. Patients routinely exposed to animals or to animal serum products can be prone to this interference and anomalous values may be observed. Additional information may be required for diagnosis. Disclaimer: Indeterminate value has been defined by Laboratory.
- HIV value between 1 to 10 S/CO should be considered as indeterminate and advised to repeat with new fresh sample.**

----- End Of Report -----

For test performed on specimens received or collected from non-NSRL locations, it is presumed that the specimen belongs to the patient named or identified as labeled on the container/test request and such verification has been carried out at the point generation of the said specimen by the sender. NSRL will be responsible Only for the analytical part of test carried out. All other responsibility will be of referring Laboratory.

Note:(LL-VeryLow,L-Low,H-High,HH-VeryHigh ,A-Abnormal)

Amit Prajapati

Dr. Amit Prajapati

DCP.(G-17008)

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